

Dementia: additional observations

Kevin Somerville, HCF May 2015



What we were told is of importance to claims consultants!

- Definition of dementia/AD for CI
- Income protection being extended to older lives aged 70 or 70 plus. Is dementia a likely problem?
- Dementia as an early benefit trigger for draw down in a whole of life policy: the trigger is ADL failure rather than the diagnosis

Alzheimer's disease/Dementia – *resulting in permanent symptoms*

A definite diagnosis of Alzheimer's disease by a consultant neurologist, psychiatrist or geriatrician.

There must be permanent clinical loss of the ability to do all of the following:

remember;

reason; and

perceive, understand, express and give effect to ideas. For the above definition, the following is not covered:

- other types of dementia.

A definite diagnosis of dementia by a consultant neurologist, psychiatrist or geriatrician.

The diagnosis must be supported by evidence of progressive loss of ability to do all of the following:

remember;

to reason; and

to perceive, understand, express and give effect to ideas. For the above definition, the following is not covered:

Dementia secondary to alcohol or drug abuse.

..... and Creutzfeldt-Jakob disease

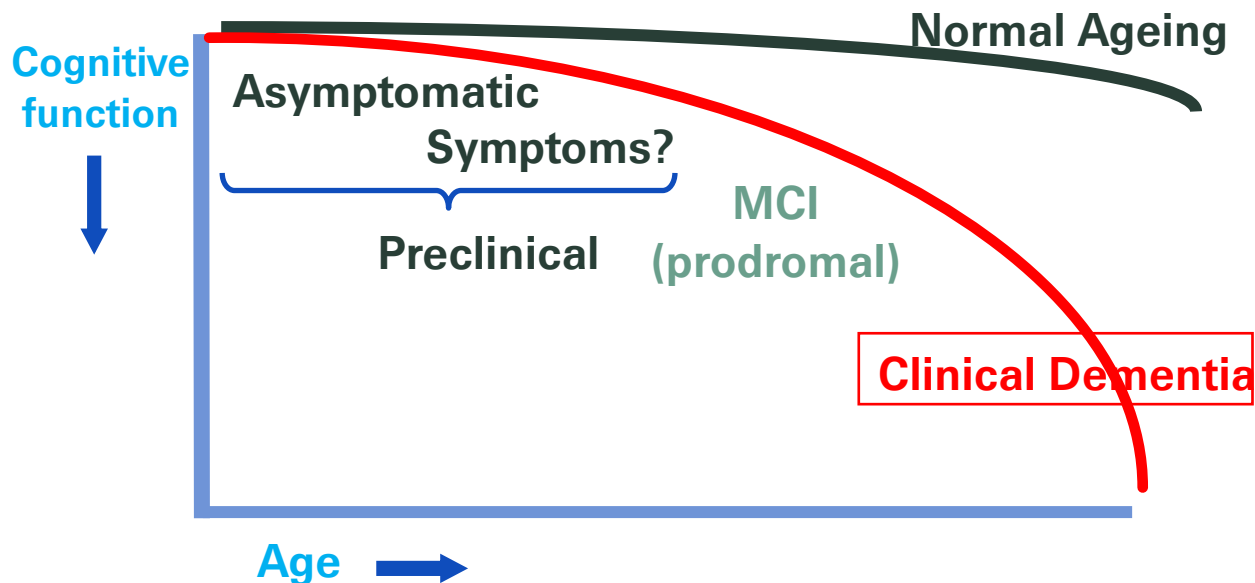
- A definite diagnosis of Creutzfeldt- Jakob disease made by a consultant neurologist. There must be permanent clinical loss of the ability in mental and social functioning to the extent that permanent supervision or assistance by a third party is required.

Diagnosis of dementia

- Clinical syndrome with cognitive decline from the previous level of performance affecting
 - multiple domains (DSM IV; ICD-10 AD): memory, judgement, reasoning etc
 - one or more domains (DSM 5: major neurocognitive disorder)
- Interference with everyday activities (AADLs; IADLs) is characteristic and invariable (cf MCI)
- Loss ADLs later; behavioural issues may predominate
- No imaging gold standard
- Cognitive testing helps but no scoring cut-off like BP or BMI (these are arbitrary anyway!)
- Considerable subjective element; may depend upon informant

Problems with terminology and diagnosis

- Alzheimer's disease: neuropathological or clinical?
 - CAD vs IHD (limited analogy)
 - Can Alzheimer's disease cause MCI?
- Is sub-clinical Alzheimer's disease ubiquitous in the very old?
- At what point does MCI become dementia?



"Ten warning signs for dementia"

Memory loss that disrupts daily life

Challenges in planning or solving problems

Difficulty completing familiar tasks at home, at work or at leisure

Confusion with time or place

Trouble understanding visual images and spatial relationships

New problems with words in speaking or writing

Misplacing things and losing the ability to retrace steps

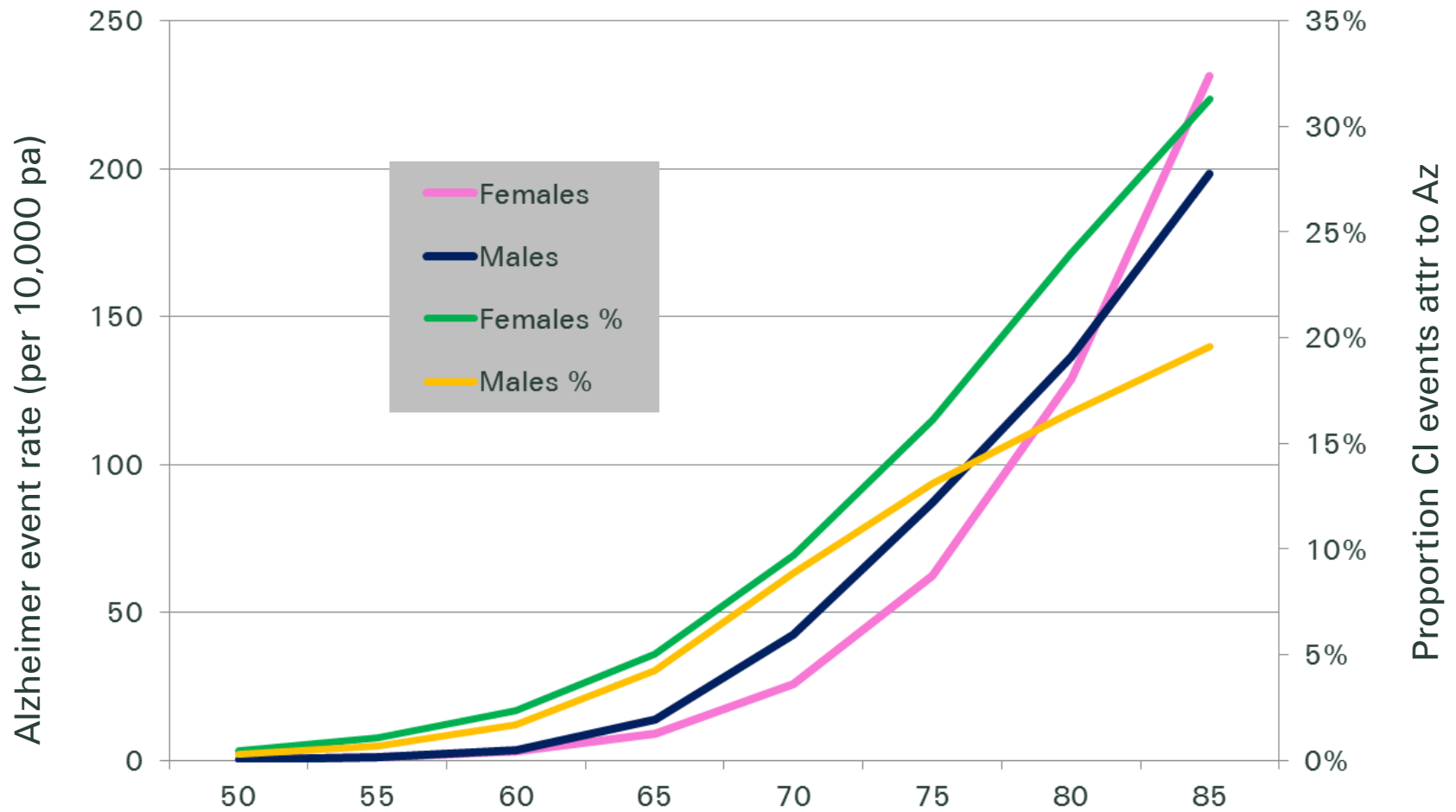
Decreased or poor judgment

Withdrawal from work or social activities

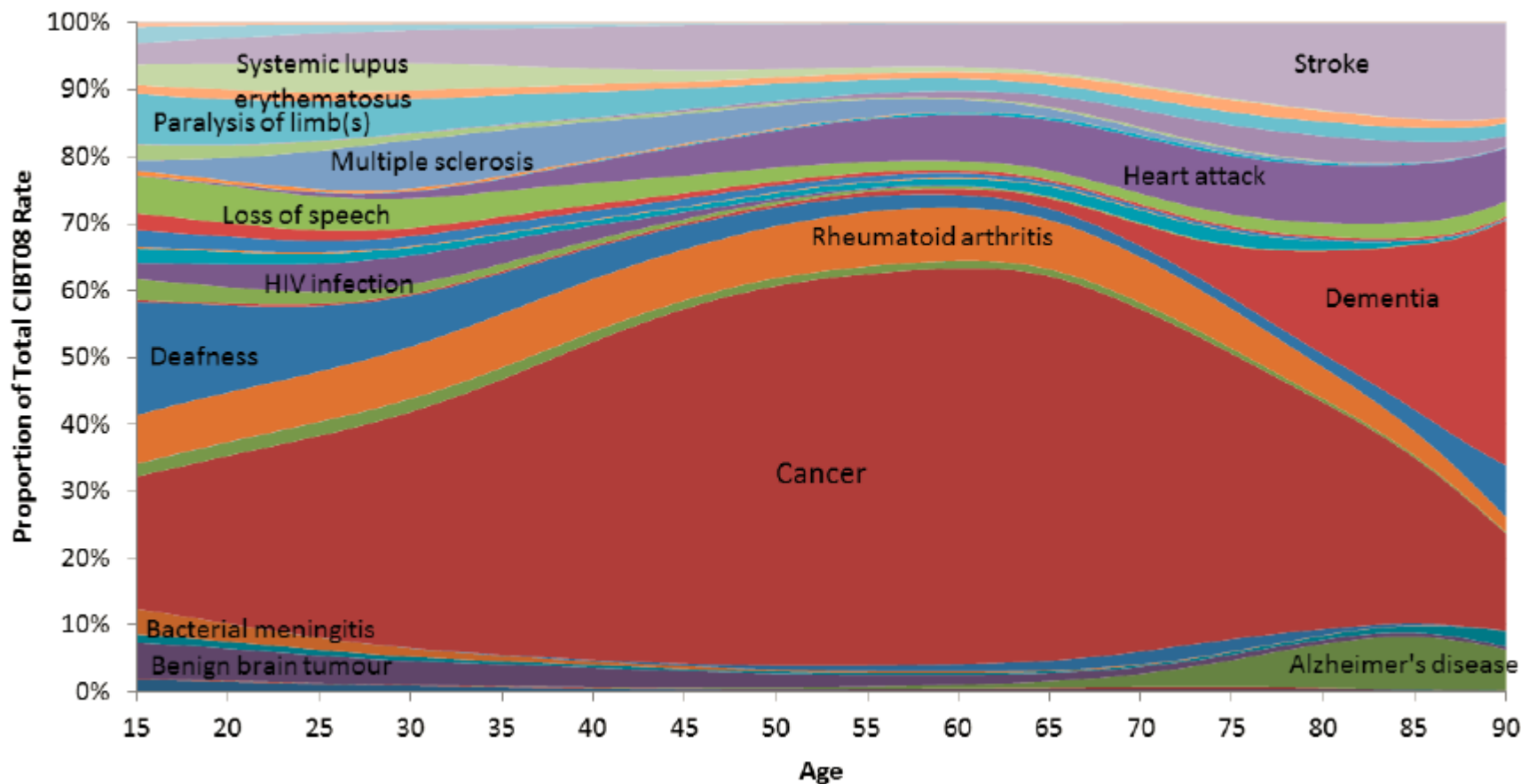
Changes in mood and personality

http://www.alz.org/alzheimers_disease_10_signs_of_alzheimers.asp#signs

CIBT 02: Alzheimer's disease



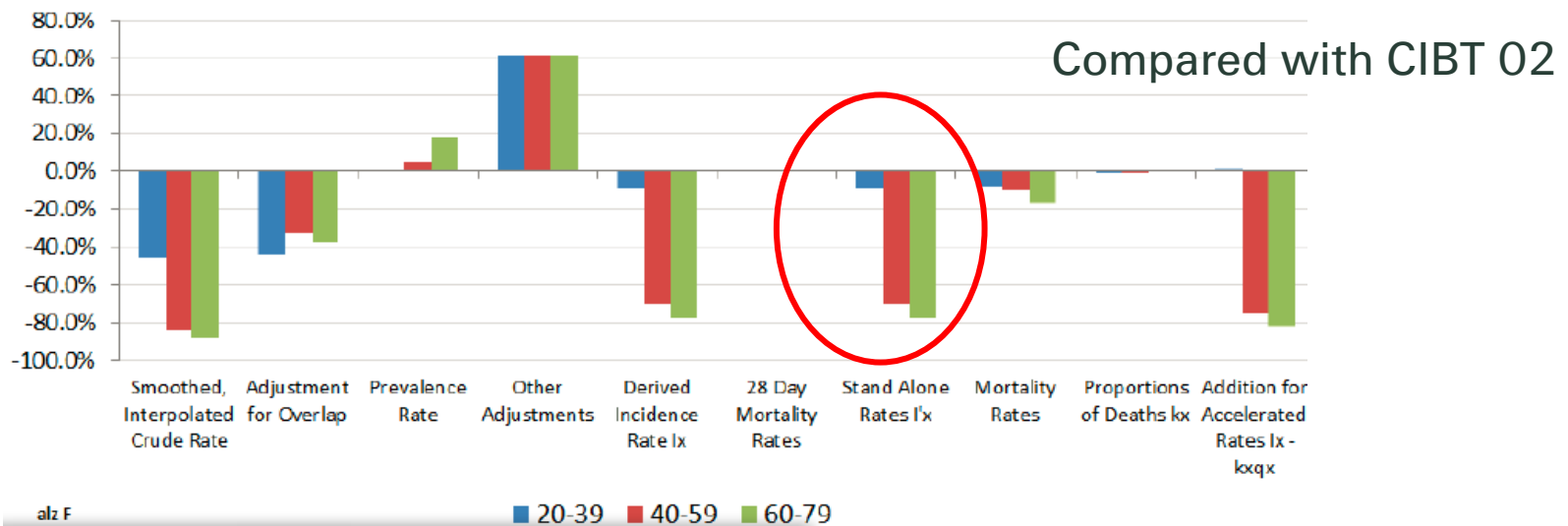
CIBT08 by Condition (Females)



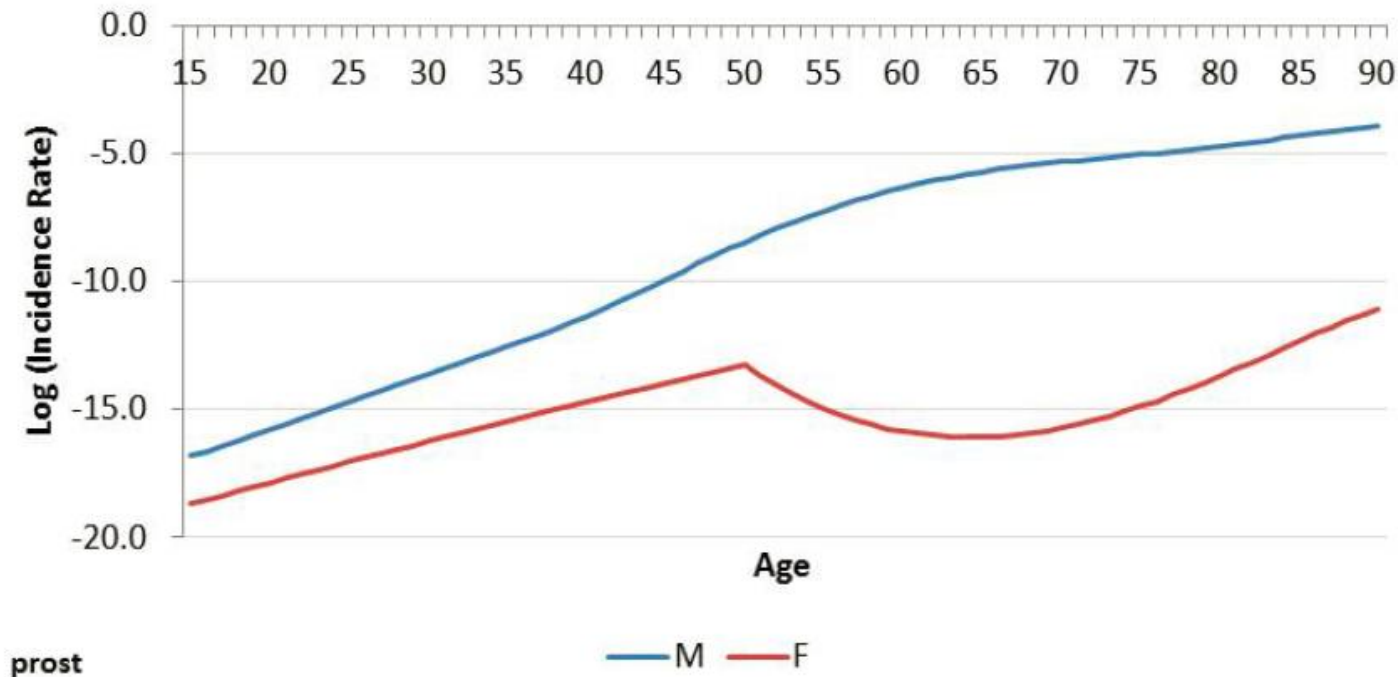
Extending the Critical Path: A report from the Critical Illness Definitions and Geographical Variations Working Party SIAS 2013

Alzheimer's disease: derived incidence rates in CIBT 08

- "Our Alzheimer's rates are significantly lower than those derived in Exploring the Critical Path as we would expect given the majority of Alzheimer's cases would not be treated as in-patient admissions."
- "Overall we are not comfortable with these rates as true estimates of the incidence of Alzheimer's disease. We have included them for consistency with our overall methodology and in particular because of the interaction with our first ever CI calculations. "



An actuarial reality check.....?



As expected, prostate cancer incidence rates increase materially by age. We have left the female curve in this chart but the rates are trivial as we would expect.

Extending the Critical Path: A report from the Critical Illness Definitions and Geographical Variations Working Party SIAS 2013

Clinical Dementia Rating scale

Domain	None	Questionable	Mild
Memory	No memory loss/ slight inconstant forgetfulness	Consistent slight forgetfulness; partial recollection of events	Moderate memory loss; more marked for recent events; defect interferes with everyday activities
Orientation	Fully oriented	Fully oriented or slight difficulty with time relationships	Moderate difficulty with time relationships; oriented for place at examination; may have geographic disorientation elsewhere
Judgment and problem	Solves everyday problems and handles business and financial affairs well; judgment good in relation to past performance	Slight impairment to solving problems, similarities, differences	Moderate difficulty in handling problems, similarities, differences; social judgment usually maintained
Community affairs	Independent function at usual level in job, shopping, volunteer and social groups	Slight impairment in these activities	Unable to function independently at these activities though may still be engaged in some; appears normal to casual inspection
Home and hobbies	Life at home, hobbies, intellectual interests well maintained	Life at home, hobbies, intellectual interests slightly impaired	Mild but definite impairment of function at home; more difficult chores abandoned; more complicated hobbies and interests abandoned
Personal care	Fully capable of self care		Needs prompting

GPs hit by widespread complaints from patients 'unhappy' over dementia screening

22 November 2013 | By Alisdair Stirling



Print



Email



Like

0



Tweet

0



+1

0



Comments (15)



Save

Exclusive GPs across England are facing complaints or objections from patients who 'resent being asked' about symptoms of dementia as part of the controversial case-finding DES, a Pulse survey reveals.

A snapshot poll of 501 GPs in England who are taking part in this year's DES has found that around one in five - 21% - have seen patients protest about the process, which involves asking those considered at risk if they are concerned about their memory.

The survey also shows almost two-thirds of GPs are experiencing longer waiting times at memory clinics since the introduction of the DES.

The DES - which aims to facilitate timely diagnosis and support for people with dementia - was introduced in April as part of Prime Minister David Cameron's 2012 'challenge on dementia'.

Under the DES, GPs are encouraged to make opportunistic offers of dementia assessment to at-risk patients where they consider it appropriate.

If a patient has suspected dementia, GPs are mandated to refer them to specialist services, such as a memory clinic, to confirm the diagnosis, then provide treatment if necessary and give advice and support to the patient's carer.

But many GPs opposed the introduction of the DES, warning at the time that it could cause undue anxiety in patients and that support services would be stretched, with some [launching a petition](#).

As part of the recently announced 2014 GP contract deal, NHS England said the DES would be changed to allow 'greater professional judgment in which patients will be offered assessment to detect possible dementia', while the BMA said there will be a new requirement to offer advanced care planning to patients who are diagnosed with dementia.

<http://www.pulsetoday.co.uk/clinical/more-clinical/elderly-care/gps-hit-by-widespread-complaints-from-patients-unhappy-over-dementia-screening/20005138.article>

GPs record 25% hike in dementia cases since introduction of 'cash for diagnosis' scheme

16 April 2015 | By [Caroline Price](#)



Print



Email



Like

0



Tweet

1



g+1

0



Comments (15)



Save

The Prime Minister has almost achieved his target to radically increase the number of people diagnosed with dementia, after the controversial £55 payment to GP practices for new cases prompted a 24% hike in recorded diagnoses.

The figures, released by the Health and Social Care Information Centre and clarified by NHS England today, show the number of patients with a record of dementia on the QOF register went up from 336,445 at the end of September - when the scheme was introduced - to a total of at least 415,964 at the end of March this year.

GPs have come under pressure since 2013 to identify more people with dementia since the Prime Minister's Challenge on dementia [pledged to get two-thirds of the population thought to have the condition formally diagnosed by 2015](#).

NHS England has now told Pulse that its new estimate for people living in England with dementia was 659,000, having changed the way it calculates [new dementia prevalence from this month](#).



RELATED ARTICLES

► [Seeing same doctor 'delays' cancer](#)



Dementia: a diagnostic battleground

Cash for Diagnoses

It is outrageous that publicly funded professionals could withhold important information about my health. It's all very well being professionally insulted by the government's offer of fifty quid. The sooner someone sues a GP for failure to diagnose as early as possible, the better. Significant financial and emotional damage, and unnecessary disability can be avoided if you are told what's wrong at the earliest opportunity and then take action to slow the progress of the disease. Recently, in London, I've seen figures for 20 per cent diagnosis rates in some practices against 75 per cent in Belfast. Francis says 'presumably' the government thought low diagnosis was evidence of poor care. If anything, it is evidence of no care at all. But that will be for the lawyers to prove.

June Andrews

Director of the Dementia Services Development Centre at the University of Stirling, March 2015

<http://www.lrb.co.uk/v37/n06/letters#letter3>

Press release

Recommendation against national dementia screening

From: [Public Health England](#)
First published: 14 January 2015
Part of: [Dementia, National Health Service, Public health and Social care](#)

The UK National Screening Committee (UK NSC) has today upheld its recommendation against screening everyone aged 65 and over for dementia.



Following an extensive review of the evidence, the Committee concluded that the current test for dementia, which is a form of questionnaire, does not accurately identify those people who have dementia and those who do not.

To recommend screening, the Committee would need to be confident that by acting early, treatments would slow or even prevent this serious disease. At the moment these treatments do not exist.

Cognitive Impairment: what else could it be?

- Depression
- Mild cognitive impairment
- Treatable cognitive changes? eg NPH, B12 deficiency
- Transient cognitive change associated with physical illness eg delirium

Search ON THIS DAY by
date

27

August

GO

[Front Page](#) | [Years](#) | [Themes](#) | [Witness](#)

1990: 'Guinness Four' guilty

All four defendants in the marathon Guinness trial are facing jail after the jury returned "guilty" verdicts following five days of deliberations.

Ernest Saunders, Gerald Ronson, Sir Jack Lyons and Anthony Parnes were convicted on all but one of the charges they faced.



Former Guinness Chairman Ernest Saunders was the central figure in the case

Ernest Saunders: diagnostic dilemma

EDITOR,—Three expert medical witnesses, called by the court in Ernest Saunders during the hearing of appeal against conviction in April 1991, testified that he was suffering from Alzheimer's disease. A review of newspaper reports of evidence presented to the Appeal Court, which was based on a computed tomography scan, repeat more than



Jeremy Warner: Ronson – Ernest's Alzheimer's was my idea

Saturday 06 June 2009

mistaken identification of Gerald Ford rather than George Bush as the American president, and an occasion when he had wandered off in the wrong direction after leaving Dr Patrick Gallwey's consulting room.¹⁻⁴

Dr George Perkin, a consultant neurologist called by the crown, told the court that there was no definite correlation between an abnormal brain scan and dementia and that he did not agree that Alzheimer's disease could be diagnosed in Mr Saunders's case without the results of

further psychometric tests and longer assessment. Dr Perkin considered depression, rather than dementia, to be responsible for Mr Saunders's symptoms. The diagnosis of Alzheimer's disease was accepted by the Appeal Court judges and Mr Saunders's sentence reduced.

It now seems that the diagnosis of Alzheimer's disease was certainly incorrect. Dr Gallwey had treated Mr Saunders for depression between February and November 1988, and the observed cognitive deficits were probably related to a depression as Dr Perkin suggested at the appeal. Atrophy on computed tomography may be seen in

in no way

have been
unprece-
dented
zheimers
members of
s in which

those involved have genuine Alzheimer's disease or other dementias. This case and its aftermath draw attention to the difficulties of making a clinical diagnosis of Alzheimer's disease. These are now well recognised, in that research on patients with the disease necessitates the use of internationally agreed diagnostic criteria.⁶ Though use of such criteria may not always be appropriate in routine or, indeed, forensic clinical assessment, we believe that, when a clinical diagnosis of Alzheimer's disease needs to be made with authority, it should surely be supported by recognised neuropsychological tests.

ROBERT HOWARD
SIMON LOVESTONE
RAYMOND LEVY

Institute of Psychiatry,
London SE5 8AF

<http://www.bmj.com/content/304/6841/1568.1>

